MAHATMA GANDHI UNIVERSITY, KOTTAYAM
MGU-UGP (HONOURS) REGULAR EXAMINATION March 2026- Fourth SEMESTER
Q.P.25901529 Discipline Specific Elective (DSE) - MG4DSESWK203 - **Social Work With Differently Abled**

Part A –(Very Short Answer) Answer all questions Each question carries 1 marks

1. Explain the Meaning of disabilities. -[U] / [CO2]- -1 marks

Disability refers to any physical, mental, intellectual, or sensory impairment that restricts a person's ability to perform normal daily activities. It results from the interaction between an individual's condition and environmental barriers, limiting participation in society.

2. Explain the human rights approaches -[U] / [CO2]- -1 marks

The human rights approach views persons with disabilities as equal rights holders.
a) Focuses on dignity, equality, and non-discrimination
b) Emphasizes inclusion and participation in society

3. State two causes of locomotor disability. - [K] / [CO1] -1 marks

- Congenital defects (present from birth)
- Accidents or injuries (fractures, spinal cord injury)

4. List two objectives. - [K] / [CO6]- 1 marks

General objectives related to disability welfare)

- To promote equal opportunities and social inclusion
- To provide rehabilitation and support services

5. Explain the two problems faced by differently abled person in daily living. - [U] / [CO1]- 1 marks

- **Mobility issues:** Difficulty in moving, accessing buildings or transport
- **Social barriers:** Discrimination, stigma, and lack of acceptance

6. Explain the person with disabilities education. -- [U] / [CO2]- 1 marks

Education for persons with disabilities focuses on inclusive and special education systems.

- Provides equal learning opportunities -Includes special schools and inclusive classrooms
- Uses assistive devices and adapted teaching methods

7. Identify the major approaches to disability. - [U] / [CO2]- 1 marks

- Medical Approach (focus on treatment and cure)
- Social Approach (focus on removing barriers)
- Rights-Based Approach (focus on equality and rights)
- Rehabilitation Approach (focus on functional improvement)

8. List two causes of intellectual disability. - [K] / [CO3] 1 marks

Genetic conditions (e.g., Down syndrome)
Brain injury or infections during pregnancy or early childhood

9. State the year of National Trust Act. - [K] / [CO5]- 1 marks

Year of National Trust Act 1999

10. Explain the magnitude of disability . - [U] / [CO2]- 1 marks

Magnitude of disability refers to the extent and prevalence of disability in society.

- Large number of people are affected globally and nationally -Includes -physical, mental, sensory
- Higher in developing countries due to poverty and lack of healthcare

Part B Short Answer Answer any 5 questions Each question carries 3 marks

11. Categorize the mental disability and physical disability. - [An] / [CO2] **-3 marks**

- **Mental Disability:** Includes intellectual disability, mental illness, developmental disorders affecting thinking, behavior, and learning.
- **Physical Disability:** Includes locomotor disability, visual and hearing impairments affecting body movement or senses.
- **Basis of Classification:** Mental relates to cognitive functioning; physical relates to bodily functions and mobility.

12. Analyze and write about three common disabilities with examples. - [An] / [CO1] **-3 marks**

- **Locomotor Disability:** Difficulty in movement (e.g., paralysis, amputation).
- **Visual Impairment:** Partial or complete loss of vision (e.g., blindness).
- **Intellectual Disability:** Limited intellectual functioning (e.g., Down syndrome).

13. Describe assessment process. - U] / [CO4] **-3 marks**

- **Data Collection:** Gathering medical, psychological, and social information.
- **Diagnosis & Evaluation:** Identifying type and severity of disability.
- **Planning Intervention:** Developing rehabilitation or treatment plans based on needs.

14. Explain the importance of teamwork in rehabilitation. U] / [CO6] **-3 marks**

- **Holistic Care:** Combines efforts of doctors, social workers, therapists.
- **Better Outcomes:** Coordinated services improve recovery and independence.
- **Support System:** Provides emotional, social, and medical support to the individual.

15. Describe types of cerebral palsy. - U] / [CO3] **-3 marks**

- **Spastic Cerebral Palsy:** Stiff muscles and difficulty in movement.
- **Athetoid (Dyskinetic):** Involuntary and uncontrolled movements.
- **Ataxic Cerebral Palsy:** Poor balance and coordination.

16. Criticize the problems faced by the differently abled persons in our society. - An] / [CO2] **-3 marks**

- **Social Discrimination:** Stigma, exclusion, and negative attitudes.
- **Accessibility Issues:** Lack of ramps, transport, and facilities.
- **Limited Opportunities:** Barriers in education and employment.

17. Demonstrate an insight into the mental disability. - A] / [CO1] **-3 marks**

- **Nature:** Affects intellectual functioning, behavior, and adaptive skills.
- **Causes:** Genetic factors, brain injury, or environmental influences.
- **Need for Support:** Requires special education, care, and social inclusion.

Part C Short Essay Answer any 3 questions Each question carries 5 marks

18. Show role of family support in mood disorder. - A] / [CO3] **5 marks**

- **Emotional Support:** Provides love, care, and understanding, reducing stress and isolation.
- **Treatment Adherence:** Encourages regular medication and therapy follow-up.
- **Early Identification:** Helps recognize symptoms like depression or mood swings early.
- **Stable Environment:** Creates a supportive and stress-free home atmosphere.
- **Rehabilitation Support:** Assists in social functioning and recovery process.

19. Show the role of government in implementing PWD Act. - A] / [CO5] **5 marks**

- **Policy Implementation:** Ensures laws and schemes for persons with disabilities are enforced.
- **Equal Opportunities:** Promotes education, employment, and reservation policies.
- **Accessibility Measures:** Develops barrier-free infrastructure and public services.
- **Awareness Programs:** Conducts campaigns to reduce stigma and discrimination.
- **Provision of Services:** Provides rehabilitation, financial aid, and assistive devices.

20. Show community participation in rehabilitation process. - A] / [CO6] **5 marks**

- **Awareness Creation:** Community educates and reduces stigma towards disability.
- **Support Systems:** Provides social and emotional support to individuals.
- **Resource Mobilization:** Helps in arranging funds, services, and facilities.
- **Inclusion:** Encourages participation in social, cultural, and economic activities.
- **Sustainability:** Ensures long-term success of rehabilitation programs.

21. Evaluate the challenges faced by differently abled persons. [E] / [CO2] **5 marks**

- **Social Barriers:** Discrimination, stigma, and lack of acceptance.
- **Physical Barriers:** Inaccessible buildings, transport, and infrastructure.
- **Economic Challenges:** Limited job opportunities and financial insecurity.
- **Educational Barriers:** Lack of inclusive education and special support.
- **Psychological Issues:** Low self-esteem, stress, and social isolation.

22. Analyze the major causes of disability. - An] / [CO1 **5 marks**

- **Genetic Factors:** Hereditary conditions and chromosomal abnormalities.
- **Prenatal Causes:** Malnutrition, infections, or substance abuse during pregnancy.
- **Perinatal Causes:** Birth injuries, premature birth, lack of oxygen.
- **Postnatal Causes:** Accidents, infections, and diseases.
- **Environmental & Social Factors:** Poverty, lack of healthcare, and unsafe conditions.

Part D Essay Answer any 2 questions Each question carries 15 marks

23. Analyse communication barriers. An] / [CO4] 15 marks

- **Physical Barriers:** Noise, distance, poor infrastructure, or lack of assistive devices hinder communication.
- **Physiological Barriers:** Disabilities such as hearing loss, speech impairment, or visual problems affect message exchange.
- **Psychological Barriers:** Emotions like fear, anxiety, anger, or low confidence distort communication.

➢ **Language Barriers:** Differences in language, vocabulary, or use of technical jargon create misunderstanding. ➢ **Cultural Barriers:** Variations in beliefs, values, customs, and non-verbal cues lead to misinterpretation. ➢ **Perceptual Barriers:** Individual perceptions and biases influence how messages are understood. ➢ **Attitudinal Barriers:** Negative attitudes, prejudice, or lack of interest reduce effective communication. ➢ **Organizational Barriers:** Rigid hierarchy, poor management, and lack of clear channels block communication flow. ➢ **Semantic Barriers:** Misinterpretation of words, symbols, or meanings leads to confusion. ➢ **Technological Barriers:** Lack of access to communication tools or poor connectivity affects interaction. ➢ **Information Overload:** Too much information at once reduces clarity and retention. ➢ **Lack of Feedback:** Absence of response prevents clarification and leads to misunderstanding. ➢ **Poor Listening Skills:** Inattention or selective listening disrupts effective communication. ➢ **Environmental Factors:** Uncomfortable surroundings (heat, crowd, distractions) affect concentration. ➢ **Social and Economic Barriers:** Poverty, illiteracy, and social inequality limit access to communication resources.	
24. Analyse professional challenges in rehabilitation practice. (Analyse – CO2). - **Professional Challenges in Rehabilitation Practice (15 Points)** ➢ **Lack of Resources:** Shortage of funds, infrastructure, and assistive devices limits effective service delivery. ➢ **Shortage of Trained Professionals:** Insufficient number of qualified therapists, social workers, and specialists. ➢ **Interdisciplinary Coordination Issues:** Poor teamwork among doctors, therapists, and social workers affects outcomes. ➢ **High Caseload:** Professionals often handle many clients, reducing quality of care and individual attention. ➢ **Limited Awareness:** Lack of awareness among clients and families about rehabilitation services. ➢ **Social Stigma:** Negative societal attitudes hinder acceptance and participation of persons with disabilities. ➢ **Policy Implementation Gaps:** Weak enforcement of disability laws and schemes reduces effectiveness. ➢ **Ethical Dilemmas:** Issues like confidentiality, consent, and autonomy create professional challenges. ➢ **Cultural Barriers:** Differences in beliefs and traditions may affect acceptance of rehabilitation methods. ➢ **Accessibility Issues:** Lack of barrier-free environments makes service delivery difficult. ➢ **Financial Constraints of Clients:** Poverty limits access to treatment and long-term rehabilitation. ➢ **Motivation Issues:** Clients may lack motivation due to chronic conditions or slow progress. ➢ **Technological Limitations:** Limited access to advanced rehabilitation technology and assistive tools. ➢ **Emotional Burnout:** Professionals face stress and burnout due to continuous exposure to challenging cases. ➢ **Monitoring and Follow-up Problems:** Difficulty in ensuring continuity of care and long-term support.	An] / [CO6] **15 marks**
25. Analyse social challenges. - ➢ **Social Stigma:** Negative attitudes and stereotypes lead to exclusion and discrimination. ➢ **Discrimination:** Unequal treatment in education, employment, and public life. ➢ **Lack of Social Acceptance:** Society often fails to accept persons with disabilities as equal members. ➢ **Isolation:** Limited social interaction results in loneliness and marginalization. ➢ **Accessibility Issues:** Inaccessible public spaces, transport, and services restrict participation. ➢ **Educational Barriers:** Lack of inclusive education and support systems. ➢ **Employment Challenges:** Limited job opportunities and workplace discrimination. ➢ **Poverty and Economic Dependence:** Disability often leads to financial insecurity. ➢ **Gender Discrimination:** Women with disabilities face double discrimination. ➢ **Cultural Beliefs and Myths:** Superstitions and misconceptions affect treatment and inclusion. ➢ **Lack of Awareness:** Society lacks knowledge about disability rights and capabilities. ➢ **Communication Barriers:** Difficulty in interaction due to language, sensory, or social factors. ➢ **Policy Gaps:** Weak implementation of disability-related laws and schemes. ➢ **Limited Community Participation:** Exclusion from social, cultural, and political activities. ➢ **Psychological Impact:** Social challenges lead to low self-esteem, stress, and reduced quality of life.	An] / [CO4 15 marks

26. Evaluate the disability and identity formation. **Disability and Identity Formation (15 Points)** ➢ **Concept of Identity:** Identity refers to how individuals perceive themselves and how they are recognized by society. ➢ **Impact of Disability on Self-Concept:** Disability can influence self-esteem, confidence, and personal identity. ➢ **Social Labeling:** Society often labels persons with disabilities, which may lead to stereotyping and stigma. ➢ **Stigma and Discrimination:** Negative attitudes affect identity development and social participation. ➢ **Internalization of Disability:** Individuals may internalize society's views, leading to positive or negative self-identity. ➢ **Positive Identity Formation:** With support, individuals can develop a strong, empowered identity beyond disability. ➢ **Role of Family:** Family acceptance and encouragement help build confidence and self-worth. ➢ **Peer Influence:** Inclusion in peer groups promotes social identity and belongingness. ➢ **Cultural Context:** Cultural beliefs and values shape how disability is perceived and experienced. ➢ **Education and Awareness:** Inclusive education helps in building confidence and reducing identity conflicts. ➢ **Role of Rehabilitation Services:** Rehabilitation supports independence and enhances self-identity. ➢ **Advocacy and Rights Movements:** Disability rights movements promote dignity and self-advocacy. ➢ **Intersectionality:** Identity is shaped by multiple factors like gender, class, and disability together. ➢ **Barriers to Identity Development:** Lack of opportunities, accessibility issues, and exclusion hinder identity formation. ➢ **Empowerment and Inclusion:** Social inclusion and equal opportunities strengthen positive identity formation.	-[E] / [CO2 15 marks